NEUROLOGY OUTPATIENT CLINICAL REPORT

Movement Disorders Evaluation

Patient ID	HS-PD-TEST-001	Visit Date	22 Aug 2026
Age	64 years	Sex	Male
Education	14 years	Referral	Progressive tremor and slowness

Document status: Synthetic clinical-style document created only for HealthSense AI testing. It does not describe a real person.

1. Chief Complaint

Left-hand resting tremor, slowness of movement, stiffness, and reduced walking speed for approximately 18 months.

2. History of Present Illness

The patient reports gradual onset of a left upper-limb resting tremor approximately 18 months ago. Over the subsequent year he developed increasing slowness when dressing, writing, and turning in bed. Family noticed reduced facial expression and softer speech. He reports occasional constipation and reduced sense of smell. No major head injury or stroke is reported.

3. Medical History

Hypertension and gastroesophageal reflux disease. No known prior neurological disorder.

4. Current Medications

Medication	Dose / Schedule	Indication
Carbidopa/Levodopa	25/100 mg, one tablet three times daily	Motor symptoms
Losartan	50 mg once daily	Hypertension
Pantoprazole	40 mg once daily	GERD

5. Neurological Examination

Alert and oriented to person, place and time. Mild hypomimia and hypophonia. Resting tremor of the left hand, reduced with voluntary movement. Bradykinesia with left-sided predominance. Increased rigidity at the left wrist and elbow. Mildly reduced left arm swing. Mild shuffling gait but independent. No dyskinesia observed.

6. Movement Disorder Scores

Assessment	Result	Comment
UPDRS Part III	32 / 132	Motor impairment, left greater than right
Hoehn & Yahr	2	Bilateral involvement without postural instability
MoCA	27 / 30	Mild recall/attention error; no major cognitive impairment

7. Levodopa Response

At a supervised follow-up assessment, motor performance improved following levodopa administration. The patient reports improved stiffness and movement speed after medication. No clinically significant dyskinesia was observed at the current dose.

8. Laboratory Results

Test	Result	Comment
CBC	Unremarkable	No major abnormality reported
Electrolytes	Within expected range	No significant abnormality
TSH	1.9 mIU/L	Within expected range
Vitamin B12	522 pg/mL	Within expected range

9. Brain Imaging

MRI Brain: Mild generalized age-related volume loss and mild chronic microvascular white-matter change. No acute infarct, mass, or hemorrhage.

DaT-SPECT: Reduced striatal dopamine-transporter uptake, greater on the left, consistent with presynaptic dopaminergic deficit.

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10. Clinical Impression

The presentation is consistent with a parkinsonian syndrome characterized by asymmetric resting tremor, bradykinesia, rigidity and reduced arm swing. The gradual course, asymmetric findings and reported levodopa response support a clinical impression of idiopathic Parkinson's disease. Final diagnosis and treatment decisions require specialist assessment.

11. Assessment Summary

Domain	Finding
Resting tremor	Present, predominantly left upper limb
Bradykinesia	Present, left greater than right
Rigidity	Present, left upper limb
Gait / posture	Mild shuffling gait and reduced left arm swing; independent
UPDRS	Part III = 32 / 132
Hoehn & Yahr	Stage 2
Cognition	MoCA 27 / 30
Imaging	Reduced striatal dopamine-transporter uptake on DaT-SPECT

12. Plan / Recommendations

Continue movement-disorders follow-up. Review levodopa response and adverse effects. Encourage individualized physical activity and physiotherapy assessment. Monitor gait, falls, swallowing, cognition, mood, sleep and medication response.

13. Information Not Available

Formal UPDRS Part II score: not available. Detailed non-motor symptom scale: not available. Genetic testing: not performed.

Testing expectation: disease_context = **parkinsons**. ADNI analysis should be skipped for this report.